

Chapter 58: Knowledge, Attitude and Practices of Primigravida Mothers Regarding Antenatal Care at Palakkad District of Kerala State

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Abstract

The pregnancy is considered one of the most magnificent states of every woman's life. It makes the women feel accomplishment at personal and social domains of life. The feeling of motherhood and new social responsibility incepts when the women come to know that they are conceived. The feeling is largely influenced by a number of factors in order to socio-economic conditions and prevailing health complications. Most of them are scared about the reality of being pregnant. It is the reason of lacking knowledge. A healthy pregnancy, childbirth and child rearing are largely influenced by the knowledge and practice domains. The nine months journey is really a knowledge seeking. The people's practices antenatal care is different, and also in the case of primigravida the attitude and practices are not in a proper way. The primigravida didn't have an idea about breast feeding. Each and every day is a new experience to them. It depends to their knowledge, attitude and practices. If the pillar is lacking it moves to a complete distress. So, the essential thing is to discuss the queries with experts and make a positive attitude and bring a magnificent primigravida period. This study aims to analyze the level of knowledge, attitude and practices of primigravida women regarding antenatal care at Palakkad District. The sample of the present study would be drawn from various hospitals in Palakkad district of Kerala State. The sample size for the study is 370. They were selected through systematic sampling method. In this study observed that one-fifths (19.7 percent) of the respondents have a high level of knowledge, attitude and practices of primigravida, half (53.3 percent) of the respondents have a moderate level and 26.8 percent of the respondents have a low level. A majority (42.2 percent) of the respondents have a moderate level of perception towards labour process and child preparedness. One-thirds (34.1 percent) of the respondents' self-esteem was low.

Keywords: Primigravida, Knowledge, Attitude, Practices, Pregnant, Child preparedness and Self-esteem

1. Introduction

The pregnancy is unexplainable phenomena of every woman's life. The very biological process of pregnancy has not only attached to emotions and feelings but it is the key process for the survival of generations and humanity. Pregnancy although bring pleasurable experience for most, there are circumstances where pregnancy brings psychosocial and emotional problems. The reasons for the same may vary from person to person and culture to culture and situation to situation and the feeling and experience they undergo in relation to the pregnancy. The experience of pregnancy affects and influences the couple alone or the entire family especially in the culturally intact societies. When exploring the pregnancy-related experiences of women in general, the women who were pregnant for the first time take a crucial place in the scenario due to the psychological and emotional states they undergo during the crucial period of their life. The experiences of primigravida in relation to the psychological and social domains not only impact them but it makes a potential impact on the fetus as well. Apart from these, the experiences of primigravida influence their relationship with spouse, parents, and in-laws. The state of psychological functioning during pregnancy even impacts the interpersonal domains of the person's life. The pregnancy experiences of women are highly depended on many socio-cultural, economic, familial and psychological aspects. Psychological distress is a pretty common phenomenon in primigravidas. With every woman, these experiences are different and dependent on their social and cultural

background. To understand the gravity of complexities during pregnancy in the case of primigravidas, it is very essential to extrapolate and understand each and every possible correlate that leaves its impact on pregnancy experience. The knowledge, attitude, and practices during the antenatal period are an imperative feature that impacts the pregnancy. A healthy pregnancy, childbirth, and child rearing are largely influenced by the knowledge and practice domains. At this juncture the present study has been conducted to assess the level of knowledge, attitude and practices of primigravida mothers regard antenatal care at Palakkad district of Kerala State.

2. Literature Review

The pregnancy experiences of women are influenced by socio-cultural, economic, familial, and psychological factors, while knowledge, attitude, and practices during the antenatal period significantly affect pregnancy outcomes. Abas and Fakhredeen (2017) reported that a small proportion of women had poor knowledge about pregnancy danger signs and symptoms, with knowledge being associated with socio-demographic characteristics. Esposito et al. (2015) found that only 42% of women were aware of potential risk factors during pregnancy and established a significant relationship between knowledge and antenatal practices. Sharma and Seelan (2017) reported that 12% of primigravida women had poor knowledge of breastfeeding, while 77% had average knowledge, with knowledge influenced by age, education, occupation, family type, and religion. Similarly, Patel et al. (2016) found that only 58% of women had adequate knowledge of antenatal care, which was significantly associated with age, occupation, family type, and socio-economic status. Yasodha and Hemavaty (2014) reported that 82% of primigravida women had inadequate knowledge of antenatal care. Ray, Rahaman, and Dasgupta (2015) observed inadequate knowledge of appropriate breastfeeding practices among more than half of antenatal mothers in rural West Bengal. Kumbani and McInerney (2006) found poor knowledge among primigravida women regarding obstetric complications and child-related vulnerabilities. Further, Kabakyenga et al. (2011) demonstrated that knowledge of obstetric danger signs significantly predicted birth preparedness, with knowledgeable women showing better preparedness and lower levels of worry. Overall, these studies emphasize the importance of antenatal education and awareness programmes, particularly for primigravida women, to improve knowledge, preparedness, and healthy pregnancy practices.

3. Materials and Methods

The present study aims at redounding to the existing knowledge on pregnancy, as experiences by primigravida, by adding new findings over issues that had not been given much consideration in previous researches. The knowledge, attitude and practices are very much important in primigravida life to getting a smooth and happy nine months journey. The knowledge, attitude and practices during the antenatal period are an imperative feature that impacts the pregnancy. A healthy pregnancy, childbirth, and child rearing are largely influenced by the knowledge and practice domains. At this juncture the present study has been conducted to assess the level of knowledge, attitude and practices of primigravida at Palakkad district of Kerala State.

4. Objectives of the Study

- 1) To know the level of knowledge, attitude and practices of primigravida at Palakkad District.

- 2) To study the association / relationship/variance between the knowledge, attitude and practices of primigravida and selected demographic variables

5. Research Design

For the present study, it was necessary for the researcher to highlight the knowledge, attitude and practices of primigravida and problems /issues encountered by them. Thus, in order to achieve all the abovementioned objectives, researcher has adopted **descriptive research design** for the present study.

6. Universe

Primigravida who are living in the jurisdiction of Palakkad district of Kerala are taken for the present study. 3500 primigravida women have been identified in Palakkad district of Kerala. These primigravida were considered as a universe for my present study.

7. Sampling Technique

The sample size for the present study is 370 primigravida they have been selected through systematic random sampling method. The sample size (n=370) has been calculated and selected as per (<http://www.raosoft.com>) sample size calculator which is based on Krejcie and Morgan sample table (Krejcie & Morgan, 1970) and it covered 95 percent confidence at 5 percent margin sample error.

8. Findings and Analysis

Table No: 1 Distribution of the respondents according to their perception towards Knowledge, Attitude and Practices about Primigravida

S. No.	Perception towards Knowledge, Attitude and Practices about Primigravida	No. of Respondents (n = 370)	Percentage
1.	Knowledge about Primigravida		
	Low	149	40.3
	Moderate	157	42.4
	High	64	17.3
2.	Attitude towards Primigravida		
	Low	95	25.7
	Moderate	200	54.1
	High	75	20.3
3.	Practices of Primigravida		
	Low	120	32.4
	Moderate	158	42.7
	High	92	24.9
4.	Overall level of KAP of Primigravida		
	Low	99	26.8
	Moderate	198	53.5
	High	73	19.7

The Table 1 shows that one-fifths of the respondents have a high level of perception towards knowledge (17.3 percent), attitude (20.3 percent) and practices (24.9 percent). Half of the respondents have a moderate level and more than one-fifths of the respondents have a low level of knowledge, attitude and practices of primigravida.

Table No: 2 Distribution of the respondents according to their perception towards Overall level of Knowledge about Labour Process and Child Preparedness

S. No.	Perception towards Overall level of Knowledge about Labour Process and Child Preparedness	No. of Respondents (n = 370)	Percentage
	Overall level of Knowledge about Labour Process and Child Preparedness		
	Low	137	37.0
	Moderate	156	42.2
	High	77	20.8

The Table 2 depicts that 20.8 percent of the respondents have a higher level of knowledge about labour process and child preparedness, 42.2 percent of the respondents have a moderate level and 37 percent of the respondents have a low level of knowledge about labour process and child preparedness.

Table No: 3 Distribution of the respondents according to their perception towards Overall level of Self-Esteem

S. No.	Perception towards Overall level of Self-Esteem	No. of Respondents (n = 370)	Percentage
	Overall level of Self-Esteem		
	Low	126	34.1
	Moderate	184	49.7
	High	60	16.2

The Table 3 expressed that 16.2 percent of the respondents have a higher level of self-esteem, 49.7 percent of the respondents have a moderate level and 34.1 percent of the respondents have a low level of self-esteem.

Table No: 4 Association between the Age of the respondents and perception towards Overall level of Knowledge about Labour Process and Child Preparedness

S. No	Age	Overall level of Knowledge about Labour Process and Child Preparedness			Statistical Inference
		Low	Medium	High	
1.	Overall level of Knowledge about Labour Process and Child Preparedness	n=137	n=156	n=077	$\chi^2 = 22.195$ df = 6 0.001<0.01 Highly Significant Contingency Coefficient=0.238
	18-21 years	58	38	23	
	22-24 years	32	50	22	
	25-27 years	37	32	18	
	Above 27 years	10	36	14	

H_a = There is a significant association between the age of the respondents and their overall level of knowledge about labour process and child preparedness.

H_0 = There is no significant association between the age of the respondents and their overall level of knowledge about labour process and child preparedness.

It is inferred from the table 4 that there is a highly significant association between the age of the respondents and knowledge towards labour process and child preparedness. It is concluded that age of the respondents has influence the level of knowledge towards labour process and child preparedness. The contingency coefficient has explained that the strength of relationship between the age of the respondents with regard to their overall level of knowledge towards labour process and child preparedness at 23 percent.

Statistical Inference:

' χ^2 ' test was used to test the above hypothesis and it was found that there is a highly significant association between the age of the respondents and their overall level of knowledge towards labour process and child preparedness ($\chi^2 = 22.195$, $0.001 < 0.01$, Significant). It is seen from table that the calculated value of the ' χ^2 ' test is greater than the table value at the 1 percent level of significance.

Hence Research Hypothesis is accepted.

Table No: 5 Association between the Age of the respondents and perception towards Overall level of Self-Esteem

S. No	Age	Overall level of Self-Esteem			Statistical Inference
		Low	Medium	High	
1.	Overall level of Self-Esteem	n=126	n=184	n=060	$\chi^2 = 20.717$ $df = 6$ $0.003 < 0.01$ Highly Significant Contingency Coefficient=228
	18-21 years	39	56	24	
	22-24 years	27	62	15	
	25-27 years	34	43	10	
	Above 27 years	26	23	11	

H_a = There is a significant association between the age of the respondents and their overall level of self-esteem.

H_0 = There is no significant association between the age of the respondents and their overall level of self-esteem.

It is hypothesized from the table 5 that there is a highly significant association between the age of the respondents and overall level of self-esteem. It is concluded that age of the respondents has influence the level of self-esteem among the primigravida. The contingency coefficient has explained that the strength of relationship between the age of the respondents with regard to their overall level of self-esteem at 22 percent.

Statistical Inference:

' χ^2 ' test was used to test the above hypothesis and it was found that there is a highly significant association between the age of the respondents and their overall level of self-esteem ($\chi^2 = 20.717$, $0.003 < 0.01$, Significant). It is seen from table that the calculated value of the ' χ^2 ' test is greater than the table value at the 1 percent level of significance.

Hence Research Hypothesis is accepted.

Table No: 6 Association between the Domicile of the respondents and perception towards various dimensions of Knowledge, Attitude and Practices about Primigravida

S. No	Domicile	Various dimensions of Knowledge, Attitude and Practices about Primigravida			Statistical Inference
		Low	Medium	High	

1.	Knowledge about Primigravida	n=149	n=157	n=064	$\chi^2 = 17.386$ df = 4 0.001<0.01 Highly Significant Contingency Coefficient=0.217
	Rural	55	64	36	
	Urban	23	26	8	
	Semi urban	71	67	20	
2.	Attitude towards Primigravida	n=095	n=200	n=075	$\chi^2 = 17.118$ df = 4 0.002<0.01 Highly Significant Contingency Coefficient=0.210
	Rural	52	83	20	
	Urban	16	30	11	
	Semi urban	27	87	44	
3.	Practices of Primigravida	n=120	n=158	n=092	$\chi^2 = 24.817$ df = 4 0.000<0.01 Highly Significant Contingency Coefficient=0.242
	Rural	53	59	43	
	Urban	22	22	13	
	Semi urban	45	77	36	
4.	Overall level of KAP of Primigravida	n=099	n=198	n=073	$\chi^2 = 26.579$ df = 4 0.000<0.01 Highly Significant Contingency Coefficient=0.259
	Rural	51	89	15	
	Urban	14	35	8	
	Semi urban	34	74	50	

H_a = There is a significant association between the domicile of the respondents and their overall level of knowledge, attitude and practices of primigravida.

H_0 = There is no significant association between the domicile of the respondents and their overall level of knowledge, attitude and practices of primigravida.

It is hypothesized from the table 6 that there is a significant association between the domicile of the respondents and their overall level of knowledge, attitude and practices of primigravida. It is concluded that domicile of the respondents has influence the level of knowledge, attitude and practices of primigravida. The contingency coefficient has explained that the strength of relationship between the domicile of the respondents with regard to their overall level of knowledge, attitude and practices of primigravida at 25 percent. The respondents who are residence in semi-urban area have a high level of knowledge, attitude and practices of primigravida.

Statistical Inference:

‘ χ^2 ’ test was used to test the above hypothesis and it was found that there is a significant association between the domicile of the respondents and their overall level of knowledge, attitude and practices of primigravida ($\chi^2 = 26.579$, 0.000<0.01, Significant). It is seen from table that the calculated value of the ‘ χ^2 ’ test is greater than the table value at the 1 percent level of significance.

Hence Research Hypothesis is accepted.

Table No: 7 One-way Analysis of Variance among Knowledge about birth process of the respondents and perception towards Overall level of Knowledge about Labour Process and Child Preparedness

S. No	Source	SS	Df	MS	$\bar{X}$		Statistical Inference
	Overall level of Knowledge about Labour Process and Child Preparedness				G1=	14.6111	F= 21.247 0.000<0.01 Highly Significant
	Between Groups	722.101	3	240.700	G2=	13.5429	
	Within Groups	4146.226	366	11.328	G3=	9.9635	
					G4=	9.9070	

G1= Completely I have, G2= I have, G3= Slightly I have, G4= Do not know

H_a = There is a significant variance among the knowledge about birth process of the respondents and their overall level of knowledge about labour process and child preparedness.

H_0 = There is no significant variance among the knowledge about birth process of the respondents and their overall level of knowledge about labour process and child preparedness.

It is inferred from the table 7 that there is a highly significant variance among the knowledge about birth process of the respondents and their overall level of knowledge towards labour process and child preparedness. It is concluded that knowledge about birth process of the respondents has influence the level of knowledge towards labour process and child preparedness. The respondents who have completely aware about birth process had a high level of knowledge towards labour process and child preparedness.

Statistical Inference:

'F²' test was used to test the above hypothesis and it was found that there is a highly significant variance among the knowledge about birth process of the respondents and their overall level of knowledge towards labour process and child preparedness (F = 21.247, 0.001<0.01, Significant). It is seen from table that the calculated value of the 'F' test is greater than the table value at the 1 percent level of significance.

Hence Research Hypothesis is accepted.

9. Recommendations

- 1) Nurse as an educator needs to understand the various health problems among people. Nursing health personnel should have adequate knowledge and skill to educate women regarding birth preparation. To improve the knowledge of the women, nurse educators play a major role so they should plan and organize health teaching for them. So, there will be a better outcome in pregnancy. Nurses should have thorough knowledge regarding the concepts and importance of birth preparation. Only then they can educate the people on this. This teaching can be provided when women come for regular health check-up.
- 2) Video assisted child birth education is a very simple and cost-effective measure which can be used effectively and independently by the nurses for reducing anxiety, pain and improving and maternal and fetal outcome of the mothers during the normal delivery process.
- 3) A primigravida woman undergoes many physiological as well as psychological changes during her pregnancy and less aware about childbirth process due to no previous experience and lack of information. This negligence among primigravida leads to further anxiety, stress and poor delivery outcome. Nurses can educate the primigravida about childbirth preparation. Midwife plays a major role all throughout a woman's pregnancy, delivery and post- partum period. The present study revealed that primigravida are keen to know about childbirth process as they have poor knowledge.
- 4) The midwifery care programme during antenatal period should be provided during their routine antenatal follow-up to provide them a great emotional, informational and tangible support. Therefore, this intervention should be instituted as a compulsory policy for caring first time mothers in order to 192 assist these women in reducing feelings of fear, anxiety and to help them cope with intra-partum stress. However, nurses should be

trained before implementing these practices in order to understand how to provide prenatal education and support.

- 5) Nursing staff working in the community as community health nurse, health worker, skill birth attendant can help and educate primigravida from community about preparation for labour and delivery, so that they feel confident and take correct steps during preparation for labour and delivery. Traditional birth attendants and skilled birth attendants can be educated by this programme and further they can educate the antenatal mothers and promote maternal and child health in the community.

10. Conclusion

Pregnancy is the period of queries. The pregnant women need correct answers to their questions, if it is not it will affect the baby too. Pregnancy is the period of hormonal alterations. It leads to physical and biological changes. Expectant mother carries a lot of fear and apprehension regarding childbirth. The mind set is differing from person to person and also the attitude. This is a surge of intricate neuro chemical changes that take place during pregnancy and cause significant fluctuations in psychological wellbeing.

References

- [1]. Abas, A. A., and Fakhredeen, A. P. D. E. Knowledge about Danger Signs and Symptoms of Pregnant Women Attending Antenatal Care Centers in Baghdad City. *IOSR Journal of Nursing and Health Science*, 6 (4), 37-40.
- [2]. Berk, L.E. (2013). Child Development. New Jersey: Pearson Education.
- [3]. Brewer, S., Bhattacharya, S., Davis, J., Meredith, S. and Preston, P. (2011). The Pregnant Body Book. London: Penguin.
- [4]. Esposito, G., Ambrosio, R., Napolitano, F., and Di Giuseppe, G. (2015). Women's knowledge, attitudes and behavior about maternal risk factors in pregnancy. *PloS one*, 10(12), 0145873.
- [5]. Kabakyenga, J.K., Östergren P.O., Turyakira E and Pettersson, K.O. (2011) Knowledge of obstetric danger signs and birth preparedness practices among women in rural Uganda. *Reproductive Health*, 8, 33-10.
- [6]. Kumbani, L.C. and McInerney, P. (2006). Primigravidas knowledge about obstetric complications in an urban health centre in Malawi. *Curationis*, 29 (3), 41-9.
- [7]. Orshan, A.S. (2008). Maternity, Newborn and NewYork: Wolter/ Lippincott William and Wilkins.
- [8]. Patel, B.B., Gurmeet, P., Sinalkar, D. R., Pandya, K. H., Mahen, A., and Singh, N. (2016). A study on knowledge and practices of antenatal care among pregnant women attending antenatal clinic at a Tertiary Care Hospital of Pune, Maharashtra. *Medical Journal of Dr. DY Patil University*, 9(3), 354.
- [9]. Ray, D., Rahaman, A and Dasgupta, A. (2015). Breastfeeding Knowledge among Antenatal Mothers: A Cross-Sectional Study in a Rural Area of West Bengal. *IOSR Journal of Dental and Medical Sciences*, 14 (5), 93-97.
- [10]. Sharma, E., and Seelan, S.G. (2017). Knowledge Regarding Breastfeeding among Primigravida Mothers. *International Journal of Recent Scientific Research*, 8 (11), 21517-21521.
- [11]. Yasodha, V and Hemavathy, V. (2014). The Knowledge of Primigravida Mothers Regarding Antenatal Care in Selected Rural and Urban Areas. *International Journal of Innovative Research in Science, Engineering and Technology*, 3 (11), 17248-17251.